NCCP 00776 · VERSION 3B

Trastuzumab deruxtecan (Enhertu) · every 3 weeks

One infusion on Day 1 of a 21-day cycle. The next infusion is normally three weeks later. A delayed appointment is rescheduled by the team; patients do not calculate catch-up doses.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Reference timing, not booked appointments. Ask who requests each test, when it is due, who reviews the result and whom to contact if it is missing. Delays can change cycle numbers; time-based monitoring remains time-based.

Check	Before starting	First cycle / month	Early treatment	Ongoing treatment	Change / completion
Blood count, kidney and liver tests	Before first dose	Before cycle 1	Before every 21-day cycle	Before every cycle; extra tests for symptoms/results	Repeat after treatment holds or toxicity as directed
Echo / MUGA	Baseline; eligibility LVEF ≥50%	Use baseline result to confirm readiness	Week 12 ≈ before cycle 5 if uninterrupted	Every 12 weeks: week 24 ≈ cycle 9; 4–8 weeks if impairment	New threshold-level LVEF decline: repeat within 3 weeks; cardiology plan
Lung symptoms / CT if suspected ILD	Review prior ILD and baseline symptoms	Explain alert card and urgent contact	Ask at each contact; no symptom-free guaranteed window	New cough, breathlessness or fever → prompt assessment/CT	Do not rechallenge after symptomatic drug-related ILD
Pregnancy / reproductive care	Pregnancy test where relevant; fertility and contraception discussion	Continue contraception	Repeat pregnancy testing if clinically indicated	Continue agreed precautions	Contraception after final dose: women ≥7 months; men ≥4 months
DEXA / tumour markers	Only if another treatment or individual indication warrants them	No automatic Enhertu-specific schedule	Use only if clinically indicated	Do not substitute markers for clinical/scan assessment	Follow the individual bone-health and disease plan
Response imaging / markers	Record measurable sites and baseline imaging	Agree first response review	Usually every 2–4 months; individualise	Adapt to disease pace; earlier for symptoms	Markers supplement, never replace, assessment

THE WHOLE TREATMENT JOURNEY

Trastuzumab deruxtecan (Enhertu) · every 3 weeks: the whole pathway

Unresectable or metastatic HER2-positive or HER2-low breast cancer, within the NCCP prior-treatment criteria

One infusion on Day 1 of a 21-day cycle. The next infusion is normally three weeks later. A delayed appointment is rescheduled by the team; patients do not calculate catch-up doses.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First cycle / month	Early treatment	Ongoing treatment	Change / completion
Treatment NCCP	Confirm exact indication and biomarkers	Usual starting dose 5.4 mg/kg intravenously on Day 1 every 21 days; prescribed reductions are 4.4 then 3.2 mg/kg	One infusion on Day 1 of a 21-day cycle. The next infusion is normally three weeks later. A delayed appointment is rescheduled by the team; patients do not calculate catch-up doses.	Continue with safety / benefit review	Treatment continues while it controls the cancer and side effects remain acceptable. This advanced-disease regimen has no automatic one-year completion date. A good scan does not by itself instruct a treatment break.
Surgery / radiotherapy ESMO_MBC2023	Only for an agreed local-control or symptom need	Teams decide sequence / any treatment interruption	No routine surgery or RT implied	Review any local-treatment effects	Reassess if goals or symptoms change
Safety checks NCCP · SMPC	Baseline bloods and individual risks	Early checks from the monitoring table	Regimen-specific checks continue	Review results before continuation	Resolve abnormalities / agree next plan
Scans / follow-up ESMO_MBC2023	Confirm baseline and treatment setting	Agree the next imaging milestone	Use the matching surveillance / response plan	Review benefit and symptoms	Continue setting-specific follow-up

What each check is for

1 **Blood tests**

Before the first dose: full blood count, kidney and liver tests, an echocardiogram or MUGA scan to measure heart pumping function, and a pregnancy test where relevant. The team checks the HER2 result, previous treatment, lung history and current breathing symptoms, and agrees how disease response will be assessed. Blood count, kidney and liver tests before each 21-day cycle. Review breathing, fatigue, nausea, intake and other symptoms at each visit. Repeat cardiac imaging every 12 weeks; assess new breathing symptoms immediately rather than waiting for the next planned scan.

Sources: NCCP · SMPC

2 **Heart checks**

NCCP specifies echo/MUGA before treatment and every 12 weeks. With uninterrupted three-week cycles, week 12 is approximately before cycle 5 and week 24 before cycle 9. Delays do not reset the elapsed-time monitoring clock. Signs of cardiac impairment can require 4–8-weekly checks; an abnormal result meeting the treatment-hold criteria requires repeat assessment within three weeks.

Sources: NCCP · SMPC

3 **Imaging**

Response assessment in advanced breast cancer is generally every two to four months, adapted to disease dynamics, sites, symptoms and treatment. Less frequent monitoring may suit indolent disease. New symptoms or suspected progression need earlier assessment. The team chooses the imaging method; this is not an automatic three-month CT chest/abdomen/pelvis order.

Sources: ESMO_MBC2023

4 **Bone health**

Enhertu alone does not require a routine bone-density scan schedule. DEXA, bone medicines or bone scans are added for another treatment, fracture risk, symptoms or disease assessment when indicated.

Sources: NCCP · SMPC

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.